

PSO Level 3 Data Expansion Initiative **FAQ**

Date of last revision: March 28, 2025

Q1. Has the PSO activity which has occurred to date at level 3 sites been reported to OH-CCO?

- No, this is the first time OH-CCO has requested PSO activity data from level 3 sites.

Q2. Does ALR reporting begin when systemic therapy begins?

- No, ALR records all cancer system activity from the time that a patient is registered with a site, which may include systemic therapy for a level 3 site, but also any pre-treatment activity including consultations with medical oncologist, radiation oncologist, haematologist, etc.

Q3. If a patient is referred to different psychosocial services (e.g., dietician, social worker) at different times, should each referral be reported separately?

- Yes, each referral to different psychosocial services should be reported separately with the respective referral dates.

Q4. How will the wait time for a second referral to a different PSO provider (e.g., social worker) be calculated, given that both providers share the same PSO program code?

- While we don't plan to calculate wait times right away, each PSO provider can have their own distinct "HCP Number" or can be grouped to a generic "HCP Number" by discipline. The "HCP Number" reported in the Clinic Visit entity is used to link to the HCP entity to determine the "HCP Specialty Code" of the provider that saw the patient. The "HCP Specialty Code" for the disciplines required by PSO are as follows:

Nutritional Therapy	06000
Social Work	12000
Psychology	15000
Psychiatry	00064
Physiotherapy	03002
Speech Language Pathology	03009
Occupational Therapy	03003

Note: If using a generic "HCP Number" by discipline, one option is to use the same number as "HCP Specialty Code". For example, create/submit a net new HCP record containing "HCP Number" and "HCP Specialty Code" with "12000". "HCP First Name" and "HCP Surname" with "Social Worker".

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Q5. Are medical psychotherapy or spiritual care included in ALR reporting for PSO activities?

- Medical psychotherapy and spiritual care are not included in ALR reporting for PSO activities. If medical psychotherapy or spiritual care are part of your existing data extraction process, you can continue to submit those visits, but it won't be counted for PSO clinic visit data in ALR.

Q6. Does this new PSO data submission have an impact on the funding that hospitals receive?

- Will capturing multiple separate PSO specialties for the same patient result in additional funding?
- Does each new referral trigger separate funding, or is there only one psychosocial referral per patient for the entire course of their care?
- The new PSO data submission will not have an impact on the funding that hospitals currently receive for PSO activities. No additional funding is triggered for separate PSO specialty visits. All PSO services provided are funded as part of the Consult/re-Consult bundle in ST-QBP.
- There are no plans to change the funding model for PSO at the moment.

Q7. We sometimes refer patients to providers in the hospital Mental Health department. Should we report this activity in ALR for PSO, or does the patient need to receive PSO services in the oncology clinic?

- Only PSO services provided to patients in the oncology clinic should be reported to ALR. PSO services provided to patients outside of the oncology clinic (ie. Mental Health unit, patient referred/seen outside of hospital, etc.) would be captured under the applicable institution/master number "type" as defined by the Master Numbering System (MNS) from MOHLTC, and MIS Functional Centre, and presumably reported under another data set (ie. DAD/NACRS, OCCI, etc.) if mandated.